



Medical First Aid (MFA)

30 Practice Questions and Answers for STCW Exams

Updated for DG Shipping India Exams | Source: brightmariner.com

Practice Set 1

Q1. Which one of the following should the first provider do when helping a crew with a snake bite?

- A) Do not remove jewelry or clothing in the area of the bite
- B) If a crew is bitten on the arm, elevate the arm above the level of the heart
- C) If the snake is still there, go back and approach from another direction to offer help
- D) Suck the venom from the snake bite

Correct Answer: B (If a crew is bitten on the arm, elevate the arm above the level of the heart)

Explanation: Elevating the affected limb above the heart level helps reduce the systemic spread of venom via the lymphatic and circulatory systems. Other options like sucking venom or leaving jewelry on are medically contraindicated as they can accelerate toxin absorption or aggravate swelling.

Q2. What is the best position for a conscious casualty suffering from internal bleeding?

- A) Laying down with legs raised
- B) Lying flat down
- C) Half-sitting position
- D) Recovery position

Correct Answer: A (Laying down with legs raised)

Explanation: Laying the casualty down with legs raised (Trendelenburg position variant) promotes venous return to vital organs and improves perfusion to the heart and brain, which is critical in cases of potential internal bleeding or shock.

Q3. The correct ratio of compression to ventilation for two rescuer CPR on a child is:

- A) 5:1
- B) 5:2
- C) 15:1
- D) 15:2

Correct Answer: D (15:2)

Explanation: In pediatric resuscitation, standard international guidelines for two-rescuer CPR specify a ratio of 15 compressions to 2 ventilations. This ensures adequate oxygenation and blood circulation for the smaller physiology of a child.

Q4. Shock is best defined as:

- A) Inadequate circulation to the body's tissues
- B) Inadequate delivery of oxygen to the lungs
- C) Inadequate elimination of waste products from the body
- D) Inadequate amount of insulin

Correct Answer: A (Inadequate circulation to the body's tissues)



Explanation: Shock is defined as a critical failure of the circulatory system to provide adequate oxygenated blood to the body's tissues. If left untreated, this leads to cellular hypoxia, organ dysfunction, and eventual cardiac arrest.

Q5. When assessing the crew, you should feel for a pulse for:

- A) 3 seconds
- B) 5 to 10 seconds
- C) 15 to 20 seconds
- D) 20 to 30 seconds

Correct Answer: B (5 to 10 seconds)

Explanation: Checking for a pulse should be quick but thorough enough to be accurate; 5 to 10 seconds is the recommended window per international first-aid standards. Taking longer delays life-saving interventions, while taking less time increases the risk of a false negative.

Q6. For any victim, the correct compression rate is:

- A) At least 120 per minute
- B) At least 80 per minute
- C) At least 90 per minute
- D) 100 to 120 per minute

Correct Answer: D (100 to 120 per minute)

Explanation: International resuscitation guidelines emphasize a high-quality chest compression rate of 100 to 120 compressions per minute for all victims. This range is optimal to maintain adequate cardiac output during external chest compressions.

Q7. One sign that a bandage may be too tight after completing compress bandage is:

- A) Blue color of the skin above the bandage
- B) Swelling below the bandage and heavy pain
- C) Heartbeat increases and a dizzy feeling
- D) Swelling above the bandage

Correct Answer: A (Blue color of the skin above the bandage)

Explanation: A blue discoloration (cyanosis) indicates restricted venous return and poor distal circulation, signifying that a bandage is applied too tightly. Immediate adjustment is required to prevent tissue ischemia.

Q8. Sun rays and light reflected from a bright surface (e.g., sea) can cause damage to the skin and eyes. What is this type of burn called?

- A) Dry burns
- B) Radiation burns
- C) Cold burns
- D) Electrical burns

Correct Answer: B (Radiation burns)

Explanation: Sunburn and damage from reflected light (such as UV glare from the sea) are classified as radiation burns. These are caused by electromagnetic radiation damaging the skin and eye tissues.



Q9. The treatment of electrical burns is:

- A) Remove loose skin and apply ointment. Don't secure with a bandage
- B) Apply lotions and ointment to the injured area and secure with a bandage
- C) Place sterile dressing over the burn and secure with a bandage
- D) Break blisters and secure with a bandage

Correct Answer: C (Place sterile dressing over the burn and secure with a bandage)

Explanation: Electrical burns often involve deep tissue damage that is not immediately visible; applying lotions or breaking blisters increases infection risk. A sterile, non-adherent dressing protects the site until professional medical assessment is possible.

Q10. Asthma attacks can be triggered by:

- A) Loud music
- B) Too much fresh air activity
- C) Not keeping to the diet
- D) Nervous tension, allergy, or non-obvious cause

Correct Answer: D (Nervous tension, allergy, or non-obvious cause)

Explanation: Asthma is a complex respiratory condition that can be triggered by a wide array of environmental, emotional, or physiological stressors. Nervous tension, allergens, and infections are common triggers identified in clinical maritime medical guides.

Q11. If a person is in a state of shock, the correct thing to do is:

- A) Apply hot-water bottles to keep the patient warm
- B) Move the casualty as much as possible
- C) Be kind to the casualty and give anything to eat or drink at the first opportunity
- D) Treat and reassure the casualty and stay with the person at all times

Correct Answer: D (Treat and reassure the casualty and stay with the person at all times)

Explanation: Treating shock requires ensuring adequate blood flow to vital organs while keeping the patient calm and warm (without overheating). Staying with the casualty provides essential psychological support and constant monitoring for changes in condition.

Q12. When you suspect a casualty has fractured a bone, you should:

- A) Raise the affected portion of the body above the level of the casualty's head
- B) Massage the affected area to prevent stiffness
- C) Rinse the area with cold water
- D) Immobilize the affected area

Correct Answer: D (Immobilize the affected area)

Explanation: Immobilization is the priority in fracture management to prevent secondary damage to nerves, blood vessels, and soft tissues. Splinting the joint above and below the fracture site restricts movement and stabilizes the bone.

Q13. If the heart of a casualty has stopped, brain damage is likely to occur after:

- A) Beyond 6 minutes
- B) Beyond 20 minutes
- C) Beyond 1 minute
- D) Beyond 10 seconds

Correct Answer: A (Beyond 6 minutes)

Explanation: Once circulation stops, the brain begins to suffer irreversible damage due to oxygen deprivation within approximately 6 minutes. Rapid initiation of BLS is essential to bridge the time until professional care is available.



- A) Silvester method
- B) Mouth-to-mouth method
- C) Heath Robinson method
- D) Mouth-to-nose method

Correct Answer: B (Mouth-to-mouth method)

Explanation: Mouth-to-mouth resuscitation is the most efficient method for delivering the required volume of oxygen to a casualty's lungs. It is the global standard for Basic Life Support when external cardiac support is combined with rescue breathing.

Q15. A resuscitator is:

- A) An insulated and heated bag that is used to wrap around a casualty suffering from hypothermia
- B) An oxygen tank, with a demand valve and mask
- C) An electrical device with two paddles that can be used to restart the heart
- D) A plastic tube that fits over the casualty's throat to keep an airway open

Correct Answer: B (An oxygen tank, with a demand valve and mask)

Explanation: In a maritime context, a resuscitator typically refers to an oxygen delivery system that includes a demand valve and a face mask. This equipment provides a controlled flow of medical oxygen to assist a patient in respiratory distress.

Q16. The best way to control severe bleeding is:

- A) Raise the bleeding part above the level of the head
- B) Direct pressure over the wound
- C) Application of a tourniquet
- D) Direct pressure on a pressure point

Correct Answer: B (Direct pressure over the wound)

Explanation: Direct pressure over the wound is the primary, most effective method to stop bleeding by facilitating clot formation. It should be the first action taken before considering secondary methods like elevation or pressure points.

Q17. The type of stretcher often found on board is:

- A) The canvas pole stretcher
- B) The Hart Imco stretcher
- C) The SOLAS stretcher
- D) The Neil Robertson stretcher

Correct Answer: D (The Neil Robertson stretcher)

Explanation: The Neil Robertson stretcher is a standard piece of maritime rescue equipment specifically designed for the safe vertical or horizontal extraction of injured personnel from confined shipboard spaces.

Q18. The unconscious or recovery position is used to:

- A) Ease the pain of broken bones
- B) Minimize nose bleeding
- C) Prevent the casualty from drowning in his own vomit
- D) Correct for any spinal injury

Correct Answer: C (Prevent the casualty from drowning in his own vomit)

Explanation: The recovery position keeps the airway patent and prevents aspiration of vomit or blood should the unconscious patient retch or experience a tongue obstruction. It is a vital safety measure for any non-breathing or unconscious casualty.



- A) When the casualty turns pale and starts to go cold CPR can be stopped
- B) Until the heart starts beating or the rescuer is unable to continue because of fatigue
- C) When the casualty shows no response to CPR after 20 minutes, it is useless to continue
- D) When the casualty has fixed and dilated pupils for 15 minutes you should stop CPR

Correct Answer: B (Until the heart starts beating or the rescuer is unable to continue because of fatigue)

Explanation: Resuscitation efforts must continue until the casualty regains spontaneous circulation, a medical professional declares them dead, or the rescuer becomes physically exhausted. Premature cessation based on time or visual appearance alone is not permitted.

Q20. The Recovery Position is:

- A) The patient is seated in a position with the head kept as low as possible
- B) The patient is placed in a 'face-to-the-floor' position with arms and legs arranged in order to stabilize this position
- C) The patient is placed flat on a bed
- D) The patient is seated in an upright position and with the arms and legs arranged in order to keep this position stable

Correct Answer: B (The patient is placed in a 'face-to-the-floor' position with arms and legs arranged in order to stabilize this position)

Explanation: The recovery position, or lateral position, is used for unconscious casualties who are still breathing. This position prevents the tongue from obstructing the airway and allows fluids like vomit or blood to drain, thereby preventing aspiration.

Q21. What do you call the method used for bone-soft part injuries?

- A) REHAB-method
- B) ABC-method
- C) First Aid-method
- D) ICE-method

Correct Answer: D (ICE-method)

Explanation: ICE stands for Ice, Compression, and Elevation. It is the gold-standard protocol for managing soft tissue injuries, such as sprains and strains, to reduce inflammation and pain effectively.

Q22. You witness a man getting electric current through his body and is stuck to the dangerous area. It is impossible to switch off the current by any main switch. How do you break the current safely?

- A) Stand on dry insulating material and pull the person away with isolating material
- B) Just take the casualty in your arms and pull the person away
- C) Apply fish oil on your hands and cut the cords by the use of any metallic pliers
- D) Call the electrician immediately

Correct Answer: A (Stand on dry insulating material and pull the person away with isolating material)

Explanation: Safety of the rescuer is paramount. Using dry insulating material prevents the electrical current from grounding through the rescuer's body, protecting them from becoming a casualty themselves during the rescue.

Q23. Abdominal Thrust is a technique that involves applying a series of thrusts to the upper abdomen to force air out of a choking casualty's lungs. How do you perform this technique?

- A) Lay the casualty on a hard surface, e.g., deck, and press firmly and rapidly on the middle of the lower half of the breastbone
- B) Remove the obstruction and restore normal breathing
- C) Let the casualty grab a list and hang upside down for a period of a minimum of 5 minutes
- D) Stand behind the casualty. Clench your fist with the thumb inwards in the center of the upper abdomen. Grasp your fist with your other hand and pull quickly inwards



Correct Answer: D (Stand behind the casualty. Clench your fist with the thumb inwards in the center of the upper abdomen)

Explanation: The Abdominal Thrust (Heimlich maneuver) is performed by placing a fist between the navel and the sternum and delivering quick, inward and upward thrusts to create an artificial cough and dislodge an airway obstruction.

Q24. When acting as a watcher or lookout at a cargo hold and men below show signs of distress, what must you do?

- A) Try to rescue them yourself
- B) Raise the alarm immediately
- C) Lower additional breathing equipment
- D) Don a B.A. set and enter the space

Correct Answer: B (Raise the alarm immediately)

Explanation: A watcher must never enter a space where they suspect a hazard because they may become a casualty themselves. The primary duty is to raise the alarm to mobilize trained emergency response teams equipped for enclosed space rescue.

Q25. A severe blow to or a heavy fall on the upper part of the abdomen (solar plexus) can upset the regularity of breathing. What are the symptoms and signs?

- A) The casualty is speaking in a very loud manner
- B) The casualty may start sweating profusely and develop a fever
- C) The casualty feels very hungry
- D) Difficulty in breathing in, and the casualty may be unable to speak

Correct Answer: D (Difficulty in breathing in, and the casualty may be unable to speak)

Explanation: A blow to the solar plexus causes a temporary spasm of the diaphragm, which controls breathing. This results in significant difficulty inhaling and often renders the casualty temporarily unable to speak due to respiratory distress.

Q26. In which way may intake of poisonous material occur?

- A) By inhalation
- B) Swallowing
- C) Skin penetration and skin absorption
- D) All mentioned

Correct Answer: D (All mentioned)

Explanation: Poisonous substances can enter the human body via ingestion (swallowing), inhalation of fumes or gases, and direct contact through skin absorption or penetration (such as needle sticks or chemical burns).

Q27. Due to exposure to heat fatigue, heat stroke, and dehydration, what is the maximum recommended effective temperature (ET) for a full workload in enclosed spaces?

- A) 30.5 degreesC
- B) 27.5 degreesC
- C) 29.0 degreesC
- D) 35.0 degreesC

Correct Answer: B (27.5 degreesC)

Explanation: Maritime health guidelines state that 27.5 degreesC Effective Temperature (ET) is the safe upper limit for continuous work in enclosed spaces to prevent heat-related illness such as heat exhaustion and heat stroke.



Q28. A casualty suddenly loses consciousness and falls to the ground letting out a strange cry. The patient is pale, red-blue in the face and froth may appear around the mouth. You are witnessing a major epileptic attack. What should you do?

- A) Move the casualty into a sit-up position and put something in the person's mouth to protect the tongue
 - B) Forcibly restrain and try to wake the casualty
 - C) Loosen tight clothing, ask all unnecessary bystanders to leave, and carefully place something soft under the head.
- If the casualty is unconscious, place the person in the Recovery position
- D) Give the casualty a lot to drink and keep talking to the person at all times

Correct Answer: C (Loosen tight clothing, ask all unnecessary bystanders to leave, and carefully place something soft under the head.)

Explanation: During an epileptic seizure, the goal is to protect the casualty from injury by clearing the area and cushioning the head. Once the seizure ends, placing the person in the recovery position ensures a patent airway.

Q29. Treatment of burns and scalds depends on the severity of the injury. What is the correct thing to do for minor burns and scalds?

- A) Break blisters, remove any loose skin or foreign objects from the injured area
- B) Place the injured part under slowly running cold water for at least 10 minutes, but preferably until the pain is gone. If no water is available, use any cold, harmless liquid
- C) Remove all sticky clothing from the casualty
- D) Apply lotions, ointments, or fat to the injury

Correct Answer: B (Place the injured part under slowly running cold water for at least 10 minutes, but preferably until the pain is gone.)

Explanation: Cooling a burn under clean, cool running water is the most effective way to dissipate heat and limit tissue damage. Applying lotions or fats traps heat and can cause further damage to the wound.

Q30. When acting as a watcher or lookout at a cargo hold and men below show signs of distress, what must you do?

- A) Raise the alarm immediately
- B) Try to rescue them yourself
- C) Lower additional breathing equipment
- D) Don a B.A. set and enter the space

Correct Answer: A (Raise the alarm immediately)

Explanation: The watcher's primary responsibility is to maintain communication and raise the alarm immediately if the workers below show signs of distress. Entering the space without the appropriate training and equipment is strictly forbidden.